



PROPOSAL FOR

IMPACT ASSESMENT OF PROJECTS

UNDERTAKEN BY

SASAKAWA-INDIA LEPROSY FOUNDATION

(S-ILF)

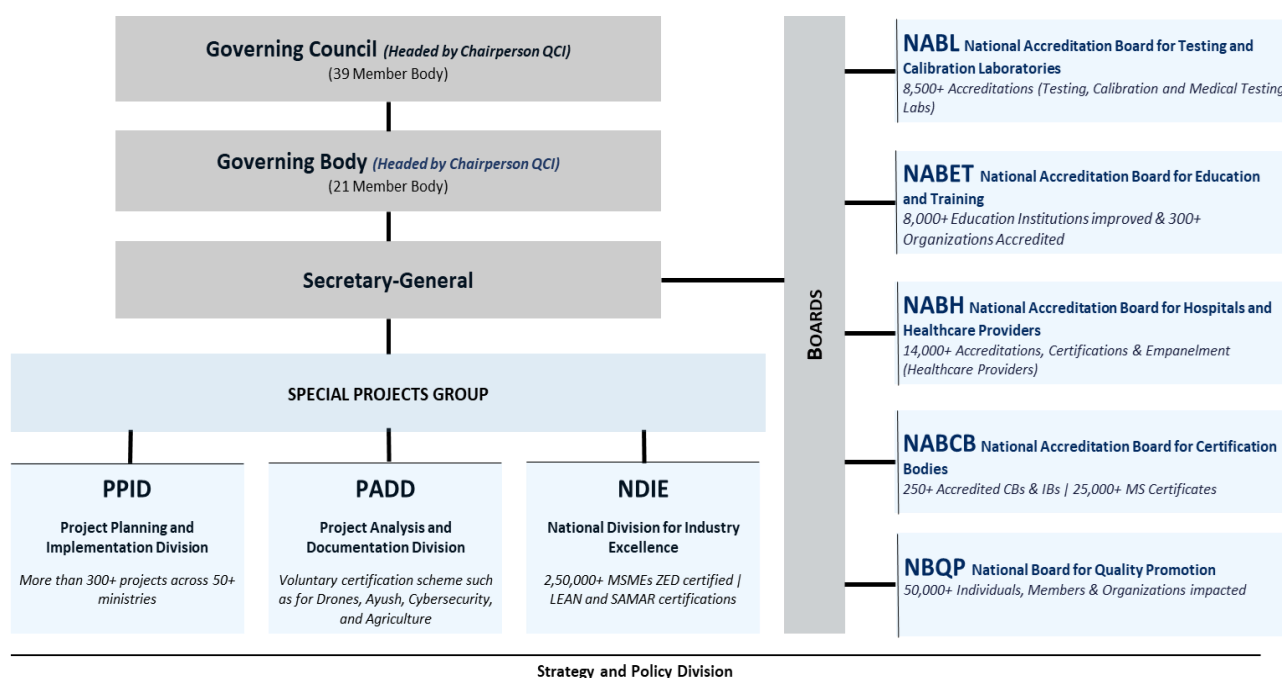
PREPARED BY:

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1. About QCI

Quality Council of India (QCI) has been set up as a non-profit autonomous society to establish and operate the national accreditation system in the country and to spread quality movement in India by undertaking a national quality campaign. It was formed in 1997 by a joint initiative of the Government of India and the Indian Industry associations like the Associated Chambers of Commerce and Industry of India (ASSOCHAM), Confederation of Indian Industry (CII) and Federation of Indian Chambers of Commerce and Industry (FICCI). QCI has established constituent boards, divisions, and cells to address various aspects of quality. These boards are active participants in international accreditation systems and are part of multilateral mutual recognition arrangements (MRAs/MLAs), ensuring global equivalence and recognition of conformity assessment results. It also facilitates the design, development and implementation of initiatives that leverage the strength of its accredited bodies in providing independent, impartial, and credible mechanisms for assuring quality across spheres of life. In the past 10 years, QCI is engaged with 90+ ministries and departments of the Union and State Governments in executing 400+ projects across multivariate spheres.



Constituent Boards

- National Accreditation Board for Testing & Calibration Laboratories (NABL):** NABL is a constituent board of QCI that accredits laboratories across scopes. NABL is a Mutual Recognition Arrangement (MRA) signatory member of APAC (Asia Pacific Accreditation Cooperation) and ILAC (International Laboratory Accreditation Cooperation) since 2000. NABL in the year 2016 achieved the APAC MRA signatory status for Proficiency Testing Providers (PTP) and Reference Material Producers (RMP) schemes along with extension in the existing fields of Testing, Calibration and Medical till December 2020. NABL has also obtained ILAC MRA for Proficiency Testing Providers (PTP) accreditation program (ISO/IEC 17043) in addition to existing Testing including Medical (ISO/IEC 17025 and ISO 15189) and Calibration (ISO/IEC 17025) programs.
- National Accreditation Board for Certification Bodies (NABCB):** Provides accreditation to Certification, Inspection, and Validation/Verification Bodies based on assessment of their competence as per international standards. NABCB is a full member of the International

Accreditation Forum (IAF), International Laboratory Accreditation Cooperation (ILAC) and Asia Pacific Accreditation Cooperation (APAC).

- **National Accreditation Board for Hospitals and Healthcare Providers (NABH):** Provides accreditation to hospitals and other healthcare organizations. NABH is a member of the International Society for Quality in Healthcare (IsQua) and its accreditations are internationally accepted. It is also a member of the Asian Society for Quality in Healthcare.
- **National Accreditation Board for Education & Training (NABET):** Committed to improving the quality of education, training, and skill development in India. It provides accreditation to schools, training institutions, and skill development organizations based on globally recognized standards. NABET also develops frameworks and guidelines to ensure excellence in education delivery and fosters continuous improvement. By promoting best practices and certifying institutions.
- **National Board for Quality Promotion (NBQP):** Dedicated to promoting a culture of quality across various sectors in India. It focuses on enhancing competitiveness through capacity building, certifications, and the adoption of best practices. NBQP conducts training programs, workshops, and seminars to equip professionals and organizations with quality skills while offering certification schemes aligned with national and international standards.

Special Projects Group (Cells and Divisions)

- **Project Planning and Implementation Division (PPID):** Specializes in conceptualizing, planning, and executing Government Flagship Schemes as required by various Government Ministries/Departments and strategy and Policy Projects that drive quality improvements across various sectors in India. This division focuses on delivering comprehensive solutions tailored to enhance efficiency, compliance, and innovation in line with national and organizational objectives. The PPID undertakes end-to-end project management responsibilities that include strategic planning, Monitoring and assessment, Implementation and Stakeholder Management.
- **Projects Analysis and Documentation Division (PADD):** Handles various voluntary certification schemes like the Voluntary Certification Scheme for Medicinal Plant Produce (VCSMPP), and INDG.A.P., Ready Mix Concrete Plant Certification Scheme (RMCPSC), Certification Scheme for Unmanned Aircraft Systems (UAS), etc.
- **National Division for Industry Excellence (NDIE):** Aim at enhancing the global competitiveness of MSMEs by gauging the manufacturer's maturity in several disciplines and gradually building their capabilities to become world class. The ZED Maturity Assessment Model has been conceived and structured to offer graded benchmark levels of an organization's performance through a set of standard enablers and outcomes focused on quality, environmental and other performance indices.
- **Strategy and Policy Division:** Specialized Unit complementing and augmenting the capacity of the government in policymaking, especially in the field of quality. It provides services in the following domains: (a) quality evaluation of goods, services, programmes, processes, and organizations, (b) strategic research and advisory in regulation, policy, quality, technology, and population scale transformations, and (c) knowledge development and analysis for emerging/futures policies.

2. Background

Leprosy, also known as Hansen's disease, is a neglected tropical disease caused by *Mycobacterium leprae* that affects the skin, peripheral nerves, and eyes. Although it is curable with early detection and multidrug therapy (MDT), the disease continues to pose public health challenges globally. According to the World Health Organization (WHO), around 180,000 - 190,000 new cases were reported worldwide in recent years, with the majority concentrated in countries such as India, Brazil, and Indonesia. While global prevalence has declined over decades, ongoing transmission especially in vulnerable and underserved populations remains a concern, and stigma and delayed diagnosis contribute to disability and social exclusion.

In India, Leprosy, though declared eliminated as a public health problem in 2005 as per WHO criteria (prevalence below 1 case per 10,000 population), continues to pose a significant public health challenge. As per official data it indicates a resurgence of the disease, with leprosy cases increasing by 15.7% in 2021 - 22 compared to the previous year. The number of active cases rose from 65,147 in 2020–21 to 75,394 in 2021 - 22, reaching 88,278 active cases by January 2023. States reporting the highest burden include **Maharashtra, Bihar, Uttar Pradesh, Chhattisgarh, Madhya Pradesh, Jharkhand, Odisha, and West Bengal**, highlighting regional concentration in socio-economically vulnerable areas.

State	Reported Leprosy Cases (Till January 2023)
Maharashtra	17,014
Bihar	11,318
Uttar Pradesh	10,312
Chhattisgarh	7,422
Madhya Pradesh	7,313
Jharkhand	6,184
Odisha	6,088
West Bengal	5,012

While the national prevalence rate declined to 0.45 per 10,000 population, the persistence and rise in new cases underscore gaps in early detection, surveillance, and sustained programme implementation. Recognizing these challenges, the Government of India launched the National Strategic Plan and Roadmap for Leprosy (2023 - 27), aiming for zero transmission and zero stigma, reinforcing the need for integrated medical, social, and rehabilitative interventions particularly those addressing stigma, discrimination, and long-term socio-economic inclusion of persons affected by leprosy.

Sasakawa-India Leprosy Foundation's Contribution:

The Sasakawa-India Leprosy Foundation (S-ILF) works to address both the medical and socio-economic challenges faced by persons affected by leprosy. Established in 2006, S-ILF empowers beneficiaries through initiatives aimed at livelihood support, vocational training, educational access,

and social inclusion, while also creating awareness to reduce stigma and discrimination. By providing grants, technical support, and capacity-building opportunities, and by engaging with policymakers and communities, S-ILF helps people affected by leprosy achieve dignified livelihoods, equal opportunities, and integration into mainstream society, ensuring long-term social and economic impact



3. About Sasakawa-India Leprosy Foundation (S-ILF)

The Sasakawa–India Leprosy Foundation (S-ILF) is a pioneering institution dedicated to the socio-economic integration of individuals affected by leprosy in India. Established in 2006 by Mr. Yohei Sasakawa, Chairman of The Nippon Foundation, S-ILF works to empower persons affected by leprosy to overcome stigma and achieve dignified and sustainable livelihoods. The foundation envisions a society in which people cured of leprosy live with dignity, equality, and self-reliance, without having to depend on charity or resort to begging. Its mission is to end stigma associated with leprosy and ensure equal social, economic, and cultural opportunities for affected persons and their families.

S-ILF seeks to increase social acceptability through the dissemination of scientific information, awareness campaigns, and dispelling myths and misconceptions surrounding leprosy. The foundation also focuses on creating enabling conditions for equitable access to education and economic opportunities. To achieve these objectives, S-ILF adopts a comprehensive, rights-based strategy, which includes:

- Providing grants and technical support for micro-enterprise development and self-employment.
- Facilitating access to government welfare schemes.
- Enabling vocational training and higher education through scholarships.
- Addressing Water, Sanitation, and Hygiene (WASH) challenges in leprosy communities to improve overall quality of life.
- Conducting widespread awareness initiatives to combat stigma and discrimination.
- Engaging with opinion leaders and policymakers to promote the rights, inclusion, and social integration of persons affected by leprosy.

Through this holistic approach, S-ILF has been instrumental in empowering beneficiaries to break cycles of social exclusion and poverty, achieve dignified livelihoods, and participate fully in society. By combining community-level interventions, awareness generation, and policy advocacy, S-ILF continues to play a critical role in improving the social, economic, and cultural well-being of individuals affected by leprosy across India.



3. Similar Past Experience

Impact Evaluation of Livelihood Projects for Sasakawa–India Leprosy Foundation (S-ILF, 2023)

Quality Council of India (QCI) previously undertook an impact evaluation of livelihood projects implemented by the Sasakawa–India Leprosy Foundation in the Champa district of Chhattisgarh. The assessment examined socio-economic outcomes of S-ILF’s interventions, with specific focus on livelihood enhancement, income changes, social inclusion, stigma reduction, and overall well-being of persons affected by leprosy and their families. The study employed Personal Interviews and Group Discussions to capture beneficiaries lived experiences and perceptions. The evaluation generated evidence on economic improvement, occupational shifts, psychosocial well-being, and increased community acceptance, and provided actionable inputs to support programme strengthening and future planning.

Evaluation of Six Central Sector Schemes – Ministry of Ayush

QCI undertook a comprehensive evaluation of six Central Sector Schemes implemented by the Ministry of Ayush, Government of India, namely Information, Education and Communication (IEC), Ayurgyan, Conservation, Development and Sustainable Management of Medicinal Plants (CDSMMP), Promotion of International Cooperation (PIC), Ayurswasthya, and Ayush Oushadhi Gunavatta evam Utpadan Samvardhan Yojana (AOGUSY). The evaluation assessed scheme design, implementation mechanisms, fund utilisation patterns, outputs, outcomes, and early impact. A mixed-methods approach was adopted, including document review, stakeholder consultations, field-level data collection, and beneficiary interactions. The findings supported evidence-based decision-making and informed recommendations for scheme strengthening and improved monitoring and evaluation frameworks.

Third Party Evaluation of Central Sector Schemes – Ministry of Labour & Employment

QCI undertook third-party evaluation studies of Central Sector Schemes implemented by the Ministry of Labour & Employment (MoLE). The evaluation covered the following schemes: e-Shram Portal [National Database of Unorganised Workers (NDUW)], National Career Service (NCS), Coaching and Guidance for SC/ST [National Career Service Centres for SC/ST], and the National Pension System (NPS). The scope of work included assessment of scheme relevance, effectiveness, efficiency, sustainability, impact, coherence, and equity; analysis of outputs and outcomes; review of implementation processes; and identification of gaps and good practices. The assignment adopted a mixed-methods approach involving secondary data review, stakeholder consultations, beneficiary surveys, Focus Group Discussions, and Key Informant Interviews, with reporting aligned to Government of India evaluation guidelines.

Kayakalp For Private Hospitals

QCI managed a nationwide assessment of 653 hospitals under the Ministry of Health and Family Welfare’s (MoHFW) Kayakalp initiative, showcasing its expertise in executing large-scale healthcare quality projects. The assessment utilized a digital tool equipped with features such as real-time, geo-tagged data collection and multi-tier quality checks to ensure data integrity and accuracy. Comprehensive analysis and reporting were conducted, culminating in detailed submissions to MoHFW that facilitated the certification of compliant hospitals. This project underscored QCI’s capability in delivering scalable, technology-driven quality assessments across the healthcare sector.

Artificial Limbs Manufacturing Corporation of India (ALIMCO) project

QCI conducted comprehensive third-party quality assessments at the ALIMCO Kanpur facility and its five Auxiliary Production Centres (APCs) for over 20 products such as wheelchairs, Walking sticks, Motorized tricycles, etc. The scope of work included developing detailed checklists, conducting on-site inspections of ready products, verifying production records and documentation, and assessing input materials, in-process controls, and final product quality. The assessments were conducted based on the agreed-upon sampling methodology and product specifications shared by ALIMCO. All data collected during the visits were systematically analyzed to evaluate compliance levels, identify deviations, and review emerging quality trends.

4. Project Overview

The proposed Impact Assessment of Projects Undertaken by S-ILF aims to assess the outcomes and impact of S-ILF's socio-economic interventions on the lives of people affected by leprosy across selected states in India. The study will focus on people's personal experiences and perceptions, how their livelihoods have improved, whether they feel more accepted in society, how their participation in economic activities has changed, and how confident and empowered they feel in their daily lives. This approach helps capture deep, meaningful insights into how the interventions have affected beneficiaries beyond measurable outcomes. It will cover approximately 136 projects, with the final sample size subject to revision based on project implementation and field conditions. It will be conducted across five states i.e., Madhya Pradesh, Uttar Pradesh, West Bengal, Odisha, and Bihar. This assessment will include qualitative and quantitative methods to understand the real-life changes experienced by beneficiaries. Primary data will be collected through in-depth personal interviews (PIs) and focus group discussions (FGDs), using a structured questionnaire designed to capture both individual and collective experiences of beneficiaries. The assessment will explore key aspects such as socio-economic challenges, access to opportunities, behavioral and attitudinal change, and the overall transformational impact of S-ILF's interventions, thereby generating evidence to inform program strengthening and future scale-up.

5. Objectives and Expected Outcomes

Sr. No.	Objectives	Expected Outcomes
1	To assess the socio-economic impact of ongoing S-ILF interventions initiated between April 2021 and March 2023 across selected states.	Evidence on socio-economic changes among beneficiaries attributable to ongoing project support.
2	To evaluate livelihood outcomes achieved, including income enhancement, employment status, and livelihood diversification.	Clear understanding of economic progress and livelihood stability resulting from S-ILF interventions.
3	To examine the effectiveness of interventions in reducing social stigma and improving social acceptance and inclusion.	Documented changes in community attitudes, dignity, and participation of persons affected by leprosy.
4	To capture beneficiaries' testimonials and perceptions through PIs and FGDs.	The narratives of the beneficiaries would enable awareness generation resulting in amplification of the voices of affected people.
5	To assess improvements in self-confidence, emotional health, and well-being.	Identification of psychosocial outcomes contributing to overall quality of life.
6	To identify implementation strengths, gaps, and emerging best practices to inform mid-course corrections and future planning.	Actionable evidence to support programme strengthening and informed decision-making by S-ILF.

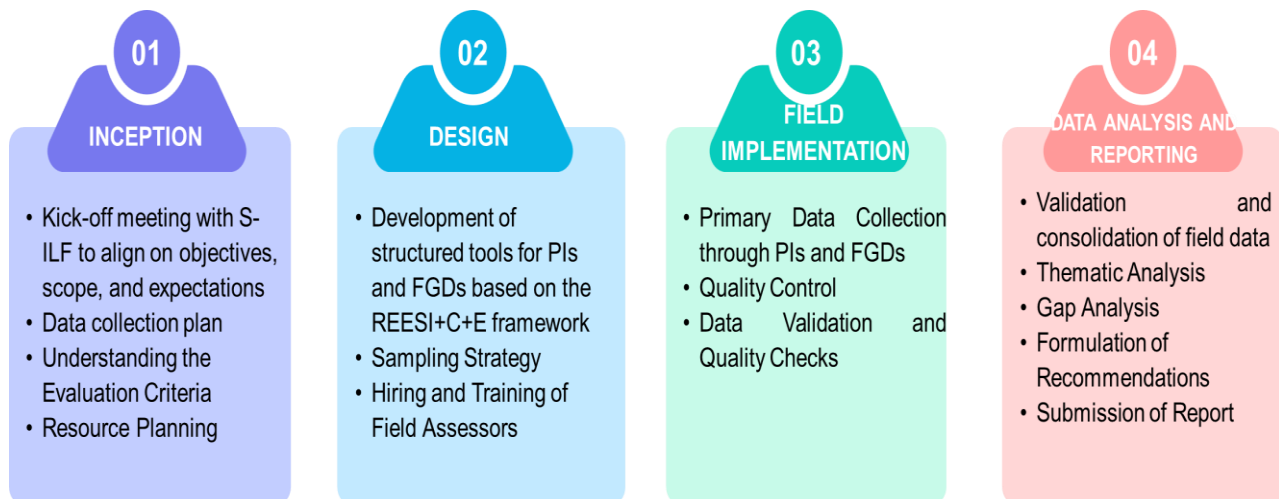
6. Scope Of Work

QCI proposes to undertake a comprehensive impact assessment and quality review of S-ILF's socio-economic interventions implemented across selected states of India. The objective of this assessment is to systematically assess the effectiveness, outcomes, and impact of projects undertaken through S-ILF, with a focus on assessing impact on livelihoods, social inclusion, economic participation, and quality of life of persons affected by leprosy. The assessment aims to generate evidence-based insights to strengthen programme design, enhance implementation strategies, while ensuring alignment with the Foundation's mission of dignity, self-reliance, and social integration.

- (i) To assess the socio-economic impact of projects supported by S-ILF, particularly in relation to livelihoods, social inclusion, dignity, and economic self-reliance of beneficiaries and their families.
- (ii) To evaluate S-ILF projects based on framework: Relevance, Effectiveness, Efficiency, Sustainability, Impact, Coherence and Equity (REESI+C+E), assessing how well the interventions respond to the needs of people affected by leprosy.
- (iii) To assess both qualitative and quantitative impacts on beneficiaries by analyzing outputs and outcomes derived from structured questionnaires across different S-ILF interventions, capturing changes at the individual and community levels.
- (iv) To identify and analyze the Most Significant Change (MSC) brought about by S-ILF initiatives by documenting transformative outcomes, beneficiary narratives, and successful implementation practices, and to highlight scalable and replicable models emerging from S-ILF projects.
- (v) To review the current implementation mechanisms and efficiency of programme delivery, identify operational gaps, bottlenecks and challenges and provide recommendations.

7. Approach & Methodology

The assessment will adopt a multi-method approach, employing a combination of tools and techniques to gather comprehensive data and insights from primary sources. Data collection methods will include structured surveys, PIs and FGDs. The methodology is organized into four sequential phases, described in detail below:



PHASE 1: INCEPTION

This phase is aimed at establishing a clear understanding of the project context, objectives, scope of the impact assessment, and stakeholder expectations. It will also set the foundation for effective coordination and execution of the assessment.

Kick-Off and Alignment with S-ILF

- Conduct an initial meeting with S-ILF to understand assessment objectives, expectations, and key focus areas.
- Review project background, implementation timelines, and geographic coverage of ongoing interventions.
- Finalize communication protocols, timelines, and reporting requirements.

Data Collection Plan

- Designing of standardized tools that will be used to collect consistent and reliable data from the field, including questionnaire for beneficiary interviews, and observation formats for direct assessment.

Understanding the Evaluation Criteria

- Outline the qualitative and quantitative metrics to be assessed.

Resource Planning

- Determine the required resources, including human resources, financials and technology needs. Establish a timeline and assign responsibilities.

PHASE 2: DESIGN

This phase focuses on developing the assessment framework, tools, and field strategy required for systematic data collection and analysis.

Development of Structured Questionnaire

- Identify key indicators and parameters aligned with assessment objectives.
- Design structured questionnaires and discussion guides for PIs and FGDs on the basis of REESI+C+E framework.

Sampling Strategy

- Finalize sampling strategy and field movement plan across Madhya Pradesh, Uttar Pradesh, Bihar, West Bengal, and Odisha.
- **Detailed sampling framework:**

State	Total Projects	Sample PIs
Madhya Pradesh (MP)	22	15
Uttar Pradesh (UP)	72	55
West Bengal (WB)	19	15
Bihar	26	15
Odisha	46	36
Total	185	136

Notes: The planned analytical sample for the assessment comprises **136 PIs across the selected states of **Madhya Pradesh, Uttar Pradesh, Bihar, West Bengal, and Odisha**.*

- In addition, 2-3 FGDs will be conducted after confirmation from S-ILF.

Hiring and Training of Field Assessors:

- Recruit and train assessors on data collection tools, ethical protocols, and field procedures.
- Conduct orientation sessions to ensure consistency and quality in data collection.

PHASE 3: FIELD IMPLEMENTATION

This phase involves field-level data collection and supervision to capture primary insights from project participants and stakeholders.

Primary Data Collection

- Conduct PIs with selected project participants across all selected states.
- Organize FGDs to capture collective experiences and group-level perspectives.

Quality Control

- Monitor field activities to ensure adherence to sampling plans and data collection protocols.

- Undertake data validation through quality checks.

Data Validation and Quality Checks

- To maintain the highest quality of data, QCI will deploy a team of QC experts and assessors.
- Team Structure: A team of QC experts based out of headquarters in New Delhi. Each expert will be responsible for overseeing the quality checks of assessors, conducting data collection.
- Reviewing Data Entries: Ensuring that data submitted by assessors is accurate and consistent with the expected standards.

PHASE 4: DATA ANALYSIS AND REPORTING

This phase focuses on consolidation, analysis, and presentation of findings to generate actionable insights for programme strengthening.

Validation and consolidation of field data

- Cleaning, checking, and organizing data to remove errors, duplicates, or inconsistencies before analysis.
- Visual Representation through Bar Charts and Histograms, Line Graphs, Scatter Plots, Heat Maps, Point and Cluster Maps will be used for data analysis.

Thematic Analysis - REESI +C+E

The assessment on thematic analysis framework based on Relevance, Effectiveness, Efficiency, Sustainability, Impact, Coherence, and Equity (REESI+C+E). This approach will allow for a structured assessment of each scheme across critical dimensions.

Gap Analysis

- Assess deviations between planned and actual implementation, including targets, timelines, and service delivery.

Formulation of Recommendations

- The assessment will assess the extent to which the objectives have been achieved and recommendations will be provided based on the key findings.
- Identification of the key problems and challenges encountered during the implementation of the objectives, and suggested appropriate policy measures and strategies for improved execution.

Submission of Report

- The report will be submitted, including an executive summary, assessment findings, and detailed recommendations, along with supporting documents, datasets, and annexures to ensure completeness and transparency.

8. Budget Estimation & Timeline

Project Timeline

PHASE	Week 1	Week 2	Week 3	Week 4	Week 5	Week 6
Inception						
Design						
Field Implementation						
Data Analysis and Reporting						

Note: The timeline is tentative and subject to change, based on the availability of documents, field conditions, stakeholder coordination, or any other operational factors.

Financials

Financials							
Headers	Line Item	Particulars	Unit	Unit cost	Qty.	Duration	Total (INR)
(A) Central Team	1	QCI Professionals	Man-Month	₹70,000	3	2	₹ 4,20,000
	2	Cost against Heading (A)					₹ 4,20,000
(B) On-Site Team	3	Assessor Cost	Man-day	₹5,000	50	1	₹ 2,50,000
	4	Cost against Heading (B)					₹ 2,50,000
(C) Logistics and Travel Cost	5	Logistics and Travel Cost					₹ 75,000
	6	Cost against Heading (C)					₹ 75,000
(D) Technology Cost	7	Report Designing					₹ 25,000
		IT Tool					₹ 10,000
	8	Cost against Heading (D)					₹ 35,000
(E) Cumulation	9	Sub-Total (SUM of Line Item 2,4,6,8)					₹ 7,80,000
	10	Admin Cost (@12% of Line Item 9)					₹ 93,600
	11	Total (SUM of Line Item 9,10) (Exclusive of GST)					₹ 8,73,600
						with GST	₹ 10,30,848.00

Terms of Payments

Milestone	Project amount to be disbursed
Letter of Award + Letter of Acceptance	30%
Completion of Field visit of proposed Samples	40%
Acceptance of Report	30%

9. General Terms

- i. **AMENDMENT OF AGREEMENT:** No amendment or modification of the agreement authorizing this project shall be valid unless it is made in writing jointly by both parties or by their authorized representatives and specially stating the reason for amendment to this agreement. The modification/changes shall be effective from the date of which they are made/executed unless otherwise agreed to.
- ii. **FORCE MAJEURE:** The parties shall ensure due compliance with the terms of this agreement. However, no party shall be liable for any claim for any loss or damage whatsoever arising out of failure to carry out the terms of the agreement to the extent such a failure is due to force majeure events such as fire, rebellion, mutiny, civil commotion, riots, strike, lockout, force of nature, accident, act of God, or any other reason beyond the control of concerned party.
- iii. **ARBITRATION:** The disputes, if arising, shall be settled by arbitration with sole arbitrator (to be jointly appointed in 30 days) in accordance with the procedure prescribed in the Indian Arbitration and Conciliation Act, 1996 or any statutory modification or re-enactment thereof and the rules made thereunder and for the time being in force. Proceedings shall, unless otherwise agreed by the parties, to be held in Delhi, India. Governing law will be laws of India. Language will be English.
- iv. **NOTICE:** All notices permitted or required to be given under the provisions of this agreement shall be deemed to be sufficiently given if written and delivered personally or sent to the other party by tele, fax or certified or registered air mail or email and addressed to the registered office of the other party as follows or to such other party address as the party may give notice in writing. Unless otherwise provided herein all such notices shall be deemed to have been given as of the date they are personally delivered or transmitted by telefax or email or deposited in the mail.
- v. **TERMINATION OF AGREEMENT:** Either party shall have the right to terminate the agreement at any time by giving thirty days' notice in writing in advance indicating reasons for same to the other party. The reasons, however, will need to be justified and in case the termination leads to delay/pause/abortion of the project, the Authority reserves the right to not make any payments further to the date of project termination as indicated by either party.
- vi. **NON-DISCLOSURE AGREEMENT:** A non-disclosure agreement shall be signed between the disclosing party and the receiving party with the purpose of preventing the unauthorized use and disclosure of confidential information.
- vii. **CONFLICT OF INTERESTS:** Both the entities shall provide a voluntary declaration of "no conflict of interest" to undermine the integrity of decision-making process and credibility